

Final Course Critical Self-Reflection

My capstone experience was one of the most meaningful parts of my graduate program because it allowed me to take a topic that was personally and professionally important to me and turn it into an applied knowledge mobilization project. When I first started this project, I was mainly focused on the fact that ovarian cancer does not have a routine population screening test. As I continued reading, meeting with my mentor, and developing my scoping review, my thinking became more focused. I realized that the main issue was not only that screening does not exist, but that this absence is not always communicated clearly to patients or even consistently understood by healthcare providers.

My most significant learning was that knowledge mobilization is not just about sharing information. It is about making information usable, clear, and connected to real decisions. In this project, that meant asking what women understand about ovarian cancer detection, what they may wrongly assume about Pap tests or pelvic exams, and what clinicians need in order to explain uncertainty without dismissing patient concerns. Through my scoping review, I learned that there is strong evidence showing routine screening with CA-125 or ultrasound is not recommended for average-risk asymptomatic women, but public understanding has not caught up with that evidence. That gap shaped the toolkit I created.

Throughout the capstone, I engaged in collaborative learning through my mentorship with Dr. Barwick and through the guidance of my academic lead. These conversations helped me think beyond simply summarizing evidence. They pushed me to ask how the findings could be translated into something practical. Earlier in the project, I was still trying to decide what the final deliverables should look like. Over time, the toolkit became more focused and intentional. I

moved from a broad idea about ovarian cancer awareness to a more complete set of resources: a patient fact sheet, a clinician conversation guide, a myth-busting webpage, and an awareness measurement tool. Each piece was created for a different audience and purpose, which helped me think more carefully about implementation.

One of my strengths during this capstone was my ability to connect evidence to a real communication problem. I was able to identify the core issue, which was the gap between clinical evidence and public understanding, and then create tools that directly responded to that gap. I also think I showed strength in making complex information more accessible. Ovarian cancer screening evidence can be difficult to explain because patients may hear “there is no screening” and feel like nothing can be done. I worked hard to frame the message in a way that was honest but still action-oriented. For example, the toolkit explains that CA-125 (tumor marker blood test) and transvaginal ultrasound are not routine screening tools, but they can be used when symptoms are present and need investigation.

At the same time, there are areas where I would still like to grow. One limitation of my capstone is that the toolkit was developed from the evidence but was not fully implemented or evaluated with patients or clinicians during the course timeline. I also found myself thinking a lot about dissemination and how challenging it can be to move a resource from being developed to actually being used in practice. Creating the toolkit was one part of the work, but I realized that getting it into the right settings, such as primary care clinics, cancer organization websites, patient education pages, or community health campaigns, would require more planning, relationship-building, and feedback from stakeholders. If I had more time, I would want to pilot the tools in a primary care or cancer organization settings and collect feedback on clarity and

accessibility. I would also want to explore translation and cultural adaptation, since ovarian cancer communication needs to be understandable for people from different language and health literacy backgrounds. This helped me see that creating a toolkit is only one step. Testing, revising, adapting, and finding realistic ways to disseminate it are just as important.

This capstone helped me grow in each of the Applied Health Sciences competencies. In evidence-based leadership, I strengthened my ability to use research and guidelines to guide practical decisions. I did not create the toolkit based on opinion. I used the scoping review findings to identify messages that were unclear or inconsistent and developed resources in response. In critical reasoning and creative problem-solving, I learned how to move from a broad issue to a more focused solution. Instead of only identifying low awareness, I thought about what kind of tools could address the communication gaps I found. In systems thinking and applied expertise, I began to see ovarian cancer detection as more than an individual patient knowledge issue. It involves public education, primary care communication, provider response, and follow-up pathways. The toolkit reflects this by including resources for both patients and clinicians. In interprofessional collaboration, I developed through mentor feedback, academic guidance, and thinking about how different healthcare roles could use the toolkit. This helped me consider the needs of both public-facing and clinician-facing audiences. Finally, in self-awareness and reflexivity, I learned a lot about how I work through uncertainty. At the beginning, I wanted the project to be strong and useful, but I was not always sure how to narrow it. Over time, I became more comfortable revising my ideas, questioning my assumptions, and recognizing my interest in patient education, cancer communication, and health system gaps.

In the next phase of my career, I hope to use the knowledge from this capstone in roles related to health communication, patient education, public health, or healthcare improvement. This project showed me that I am interested in translating evidence into resources that patients and providers can actually use. It also strengthened my confidence in working with research, identifying communication gaps, and developing practical tools. My capstone helped me grow from understanding a problem to creating a knowledge mobilization response that is evidence-informed, patient-centered, and connected to real healthcare practice.